جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : مني احمد السمان احمد
المحافظة : القاهره **الحي / المدينة :** عين شمس **العنوان :** ش البصراوى - المنيره
رقم البطاقة : 25709182100602 **رقم التليفون :** 01205496240 **تاريخ تسجيل الإستمارة :** 24-10-2026
العمر(تاريخ الميلاد) : 18-09-1957 **المهنة :** بدون عمل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام تصوير مقطعي للصدر والبطن والحوض بالصبغة [11129] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: منى احمد السمان احمد
National ID: 25709182100602

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

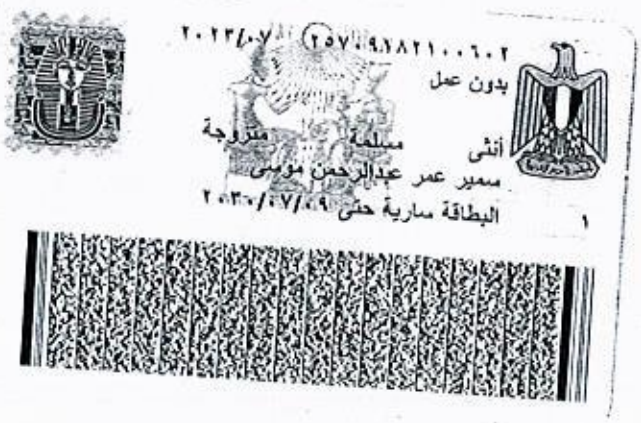
تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Contrast-enhanced CT of chest, abdomen and pelvis.**

التاريخ : **05-07-2026**	**الوقت :** **-- --:--**
	الطبيب المعالج : د/شيماء لاشين





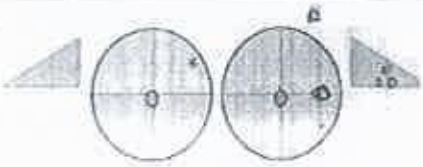
National ID - Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان

MDT form

A-Patient ID:
1- Name : منى احمد السمان احمد
Sex: Female
3- Medical number: 83239113
Old Med : 3677-2024
2-ID number:
Patient Age : 67 Y



Family history	Family History	Phone	Phone 01141116246	BMI	BMI 27	REASON OF MDT	REASON OF MDT NEW
marital status	Marital Status	NRS Score	NRS Score 2	GROUP	GROUP C	Date of MDT	Date Of MDT 09/12/2024
Co morbidity	Co Morbidity HCV	Menopausal status	Menopausal Status POSTMENOPAUSAL	Initial STAGE	Initial STAGE		

Clinical data

Breast Imaging :
lt breast malignant mass & level i,ii axillary lymphadenopathy
BIRADS 5

Mets Workup :
cap" lta xillary pathological & lt supra claviclar LN as well as few bilateral ossiptal & abd with multiple mixed lytic & sclerotic bony deposits associated with sacral soft tissue component

BX	ER	PR	Her 2 Neu	SISH	KI67
	8/8	8/8	1		6
BX COMMENT: MIXED idc+ilc	ER COMMENT:	PR COMMENT:	Her 2 Neu COMMENT:	SISH COMMENT:	

T	N	M
T2	3C	1 BONE

Sonomammography

PATIENT HISTORY
BONE SCAN

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN
FOR 1ST LINE AROMATASE INHIBITOR
PALLIATIVE RT TO RT HUMERUS
ZOMETA/28 DAYS
PAIN CLININC CONSULTATION

SIGNATURE

MEDICAL ONCOLOGY	MEDICAL ONCOLOGY د/الاء جميل	RADIOLOGY د/ايرده حكيم	RADIOLOGY
SURGERY	SURGERY	PATHOLOGY	PATHOLOGY
Psychiatric	Psychiatric	Nutrition	Nutrition

MDT Form
[[DOCTYPE:MDT_FORM]]

Name: منى احمد السمان	
MR. : 4893	

Detailed Medical Report

السيد	

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

MRI BRAIN showed a left frontal bone lesion, prompting palliative radiotherapy for chest, abdominal, and brain disease.

The patient has metastatic breast cancer to bone and has been on aromatase inhibitor therapy since December 2024. A bone scan on 16-02-2026 demonstrated very mild progression, and a CT of the chest, abdomen, and pelvis (CT CAP) is pending. The patient reported a headache but remained neurologically intact after consultation with Dr. Khaled. A repeat bone scan is planned after three months, and a methotrexate course for two weeks was administered on 20-12-2025. MRI on 20-12-2025 revealed a left frontal bone expansile infiltrative marrow signal suggestive of metastatic deposit.

A CT in March 2026 identified a calculous gallbladder with a prominent common bile duct, prompting an ultrasound and liver function tests for multidisciplinary discussion. The patient was started on Zometa and Femara.

Pharmacy records on 11-04-2026 indicate the patient received Zometa 4 mg and Femara, with weight 69 kg, age 68, serum creatinine 0.77 mg/dL, calcium 9.77 mg/dL.

On 13-05-2026, the patient was paused from therapy; no common bile duct dilatation was noted, and a bone scan was requested for Femara monitoring.

On 04-07-2026, the patient's disease status was described as post-mastectomy left breast cancer, CT2, N3, M1 with bone metastases, IDC ER8/8, PR8/8, HER2-negative, with frontal bone deposits diagnosed 11-2024. The patient was on AI therapy for new bone scans and new CTs for Zometa, Femara, and two ampules of calcium IV over 250 cc glucose 5% over 1 h.

On 01-08-2026, the patient experienced progression of ocular edema on OCT in July 2026. Femara and supportive care were planned for two months, with the next visit scheduled for 01-10-2026.

On 29-08-2026, the OCT in July 2026 showed right cystoid macular edema and left diffuse macular edema. CT CAP on 15-08-2026 revealed a left HFL 8 × 6 mm lesion and multiple axial and appendicular bony lesions. A brain MRI, dynamic liver MRI, and bone scan were ordered, and the findings were to be compared with prior CTs.

تحريرا في ,

التاريخ :	19-09-2026	**الوقت :**	

الطبيب المعالج : نهي سعيد علي محمد

يعتمد ,

Patient Name	: الأستاذه / منى احمد السمان احمد	Request No.	: 13001034
Patient No.	: 25709182100602	Request Date	: 23/10/2024
Path No.	: 4052/24	Verification Date	: 10/11/2024
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 67 Y	Department	: Out Patient
Referreing Doc.	:	IHC No.	:
		Registered by	: .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast mass.
Nature of Specimen: tRu-cut biopsy, collected at 22/10/2024.

Technique:
- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:
* IPT (Positive control showed appropriate reactivity):
- ER: Strong nuclear reaction in 90% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 90% of tumor cells (score 8/8).
- Her2: Faint incomplete membrane reaction in >% of tumor cells (score 1+).
- Ki67: 6% nuclear reaction of tumor cells.

Conclusion:
Left breast mass, Tru-cut biopsy:
Mixed invasive duct and lobular carcinoma, Grade II with minimal LCIS.
IPT:
- ER: Positive (score 8/8).
- PR: Positive (score 8/8).
- Her2: (score 1+).
- Ki67: Positive in 6% of tumor cells.

* Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0,

Verified by :
Dr. Sagda El-Kady

غرفة رقم (4) - الدور الاول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Immunohistochemistry
[[DOCTYPE:IHC]]

Patient Name	الأستاذه / منى احمد السمان احمد		
Patient No.	25709182100602	Request No.	13001034
Path No.	4052/24	Request Date	23/10/2024
Sex	Female	Verification Date	10/11/2024
Age	67 Y	Financial Category	اورام دار السلام - مبادره
Referreing Doc.		Department	Out Patient

Registered by .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10%of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Comment: Although Her2 is Negative, such cases (Score 1+) are predictive of response to Her2-Ab Drug conjugates (ADCS) according to latest 2023 ASCO-CAP guidelines.

Signature
Prof. Dr. Lobna Omar.
Dr. Menna Nabil.

Verified by :
Dr. Sagda El-Kady

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Immunohistochemistry
[[DOCTYPE:IHC]]

Name : MONA AHMED AL SAMMAN	Date : 12/11/2024
Referred by :	Patient ID : 12-11-2024MONA

MultiSlice CT Scan of the Chest, Abdomen and Pelvis using Oral and IV Contrast Revealed:

History: breast cancer

Findings:

I. Neck:

- Multiple left supraclavicular and occipital prominent lymph nodes , the largest is seen at the left supraclavicular group measuring 1.8 cm (long axis)
- Normal CT appearance of the soft tissues and fascial planes of the neck with no masses or lymph nodes enlargement.
- Normal CT appearance of the salivary glands.
- The thyroid gland is of average size and normal shape with no masses or nodules.
- Unremarkable CT appearance of the nasal cavity, nasopharynx, oropharynx and larynx.
- The upper trachea is patent.
- Intact carotid vessels on both sides.

II. Chest

- Left axillary pathological lymph nodes , the largest is measuring 1.8 cm.
- Both lungs are clear showing no consolidations, bronchiectasis, nodules or masses.
- No pleural effusion, masses or calcifications.
- No hilar or mediastinal lymphadenopathy.
- Normal CT appearance of the heart and great vessels.

III. Abdomen and Pelvis

- Few pre aortic , aorto-caval and retro caval subcentimetric lymph nodes.
- The liver is average in size showing homogenous density with no focal lesions. Patent portal vein and its main branches.
- No intra or extra hepatic biliary radicle dilatation.
- No dense stones seen in the gallbladder.
- The spleen is of average size and homogenous density with no focal lesions.
- Both kidneys are of normal site, size and shape with no stones, masses or backpressure changes.
- Small right lateral urinary bladder wall diverticulum measuring about 2.6 cm.
- No abdominal or pelvic collections or ascites.

[[DOCTYPE:CT_SCAN]]

Name : MONA AHMED AL SAMMAN	Date : 12/11/2024
Referred by :	Patient ID : 12-11-2024MONA

- Normal CT appearance of the pancreas, both supra renal glands, and bowel.
- Good opacification of the aorta and its main branches as well as the mesenteric vessels.

IV. **Musculoskeletal:**

- Multiple mixed lytic and sclerotic bony deposits noted along all examined bones with associated pathological soft tissue component at the sacral ala bilaterally measuring 8x3.9 cm partially 234encroaching on both neural sacral foramina.

Opinion

Known case of cancer breast showing left axillary pathological and left supra clavicular lymph nodes as well as few bilateral occipital and abdominal ((subcentimetric lymph nodes)) with multiple mixed lytic and sclerotic bony deposits associated with sacral soft tissue component.

Right lateral urinary bladder wall small diverticulum.

Reported by,
Dr. Mohamed A. Fouad, FEBR

[[DOCTYPE:CT_SCAN]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name. Mona Ahmed Al Samman Ahmed	Date : 22/10/2024	
Exam : ultrasound BREAST (BOTH SIDE)	Age : 67 Year(s)	
Medical ID: 25709182100602	Pat. ID: 93429	

DIGITAL BILATERAL SOFT TISSUE MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

Indication:

Left breast palpable mass

Report:

- ACR a
- Left Breast:
  - Irregular dense spiculated mass is seen in the LOQ with No overlying microcalcification measuring about 14x20mm70mm from nipple.
  - Ultrasound shows this mass as irregular heterogenous spiculated heterogeneously hypoechoic mass with internal vascularity and posterior shadow. It is seen at 4 O'clock about 9mm from the skin.
  - Multiple (>3) pathologically enlarged left level I, & II axillary lymph nodes are noted showing thick cortices and effaced hila
  - Bilateral enlarged posterior cervical rather than supraclavicular lymph nodes.
- Right Breast:
  - No suspicious micro calcifications or architectural distortion detected.
  - Complementary ultrasound no cystic or solid mass lesions.
  - Normal skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
  - Nonspecific axillary LN.

Opinion:

- Left breast malignant mass and level I & II axillary lymphadenopathy for histopathological assessment (BIRADS 5)
- Posterior cervical lymphadenopathy for biopsy
- Normal sonomammography of right breast (BIRADS 1)

Best regards,
Dr. Amira H Radwan MD,
Breast Imaging consultant,
NCI, Cairo University

Mammogram
[[DOCTYPE:MAMMOGRAM]]

Patient's Name:	**Mona Ahmed Al-Samman Ahmed**
Date:	**2026-08-15**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

Known case of CA breast.

I- CHEST:

- Left breast asymmetric densities seen for sono-mammography.
- Multiple left axillary lymph nodes largest measuring 15 x 9 mm.
- Multiple bilateral scattered atelectatic bands.
- No pulmonary nodules, masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. Left hepatic lobe segment II hypodense focal lesion measuring 8 x 6 mm. no dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows two bilateral posterolateral diverticular outpouchings. no evidence of gross masses or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- Multiple predominantly lytic bone lesions seen scattered throughout the axial and appendicular skeleton.

Opinion:

- **Left hepatic lobe hypodense focal lesion for dynamic MR study.**
- **Multiple scattered axial and appendicular skeleton predominantly lytic bone lesions likely metastatic.**

Dr.sherif Wagdy

Dr.sherif Wagdy

CT Scan
[[DOCTYPE:CT_SCAN]]

AL ROWAD

٤٨٩٢

مستشفى الرواد للعيون

Ophthalmic investigative unit

Optical coherence tomography Report

Name: Mona Ahmed **Age**: 68 y
Date: 26-7-2026
Eye: Both eyes
Referred by: Prof. Dr. Shewikar Abd el Alim
Done by: Dr Noha Khalil

Dear Sir,
Thank you very much for referring your patient,

OCT of the right eye shows:

- Altered foveal contour with altered normal foveal depression .
  - Evidence of interrupted hyperreflective epimacular membrane attached to the macula showing no vitreomacular traction.
  - Markedly significant change in thickness in the central foveal zone.
- Diffuse hyporeflective intra-retinal oedema forming cystic spaces of different sizes suggesting **cystoid macular oedema.**
- The RPE choriocapillaris complex shows irregularities and interruptions suggesting RPE changes
- Diffuse hyporeflective fluid separating the neurosensory retina from the RPE choriocapillaris complex involving the fovea suggesting neurosensory macular detachment.
- Interrupted ellipsoid zone.
- The retinal thickness map shows markedly increased central foveal thickness as compared to the age matched normative database measuring 346 microns.

OCT of the left eye shows:

- Preserved foveal contour with altered normal foveal depression .
  - Evidence of interrupted hyperreflective epimacular membrane attached to the macula showing no vitreomacular traction.
  - No significant change in thickness in the central foveal zone.
- Diffuse hyporeflective intra-retinal oedema forming cystic spaces of different sizes suggesting **diffuse macular oedema showing cystoids changes .**
- The RPE choriocapillaris complex shows irregularities and interruptions suggesting RPE changes

المركز الرئيسي (فرع الجيزة) : ٢٩ شـــارع إيـــــران – الدقـى .
فـــــرع مـــــدينة نصر : ١٦ ش عماد الدين كامل – خلف طيبة مول .
فـــــرع بنـي ســـويف : حـى الـزهور بجوار مستشفى الصـــدر .
Call 19635
E-Mail : info@alrowadhospital.com www.alrowadhospital.com

- Intact ellipsoid zone.
- The retinal thickness map shows within normal central foveal thickness as compared to the age matched normative database measuring 261 microns.

Impression:

Picture suggestive of **right cystoid macular oedema involving the fovea and left diffuse macular oedema showing cystoids changes and right neurosensory macular detachment involving the fovea** for follow up and clinical correlation. However, there is progression of the condition.

Sincerely,
Noha Khalil

Patient's Name:	**Mona Ahmed Al-Samman Ahmed**
Date:	**2026-08-15**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

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Opinion:

- **Left hepatic lobe hypodense focal lesion for dynamic MR study.**
- **Multiple scattered axial and appendicular skeleton predominantly lytic bone lesions likely metastatic.**

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AL ROWAD

٤٨٩٢

مستشفى الرواد للعيون

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Optical coherence tomography Report

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المركز الرئيسي (فرع الجيزة) : ٢٩ شـــارع إيـــــران – الدقـى .
فـــــرع مـــــدينة نصر : ١٦ ش عماد الدين كامل – خلف طيبة مول .
فـــــرع بنـي ســـويف : حـى الـزهور بجوار مستشفى الصـــدر .
Call 19635
E-Mail : info@alrowadhospital.com www.alrowadhospital.com

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Sincerely,
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Known case of CA breast.

I- CHEST:

- Left breast asymmetric densities seen for sono-mammography.
- Multiple left axillary lymph nodes largest measuring 15 x 9 mm.
- Multiple bilateral scattered atelectatic bands.
- No pulmonary nodules, masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. Left hepatic lobe segment II hypodense focal lesion measuring 8 x 6 mm. no dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows two bilateral posterolateral diverticular outpouchings. no evidence of gross masses or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- Multiple predominantly lytic bone lesions seen scattered throughout the axial and appendicular skeleton.

Opinion:

- **Left hepatic lobe hypodense focal lesion for dynamic MR study.**
- **Multiple scattered axial and appendicular skeleton predominantly lytic bone lesions likely metastatic.**

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